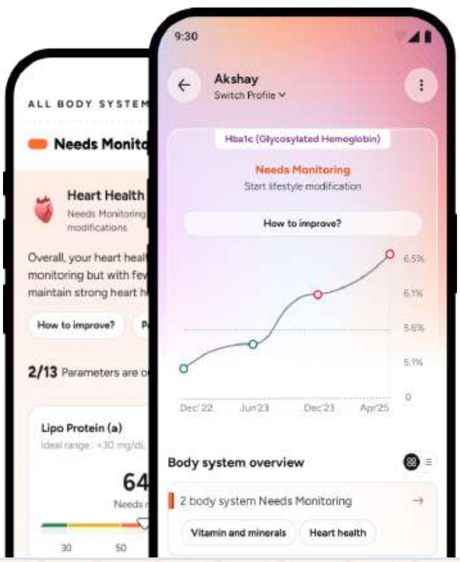
# PERSONAL HEALTH SMART REPORT

View detailed health insights, and trends on Tata 1mg app.

Click here



# Doctor Summary

Pr3(C-Anca),Hla B27 (HumanLeukocyte Antigen B27) - Pcr, Angiotensin Converting Enzyme, Anti-Nuclear Antibody (Ana) By Ifa - End Point Titer, Mpo (P-Anca) +11 More

*Note* This is an electronically generated **summary of clinically significant parameters of the attached report.** It is advised to read this summary in conjunction with the attached report and to correlate it clinically. For the trends section, the out-of-range values are highlighted with respect to the bio-reference range of respective reports.

| Test Name | Result | Bio. Ref. Interval |
|---|---|---|
| Complete Blood Count | 18 Feb 26 | |
| Hemoglobin | 12.4 g/dL | 12.0 - 15.0 |
| **RBC** | **5.12** mili/cu.mm | 3.8 - 4.8 |
| HCT | 37.9 % | 36 - 46 |
| **MCV** | **74.1** fL | 83 - 101 |
| **MCH** | **24.2** pg | 27 - 32 |
| **RDW-CV** | **17.5** % | 11.5 - 14 |
| Total Leucocyte Count | 7.10 10^3/ÂµL | 4 - 10 |
| Neutrophils | 59 % | 40 - 80 |
| Lymphocytes | 28 % | 20 - 40 |
| Monocytes | 6 % | 2 - 10 |
| **Eosinophils** | **7** % | 1 - 6 |
| Basophils | 0 % | 0 - 2 |
| **Absolute Basophil Count** | **0** 10^3/ÂµL | 0.02 - 0.1 |
| Platelet Count | 328 10^3/ÂµL | 150 - 410 |

**Trends** (For last three tests)

Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!

# Doctor Summary

Pr3(C-Anca),Hla B27 (HumanLeukocyte Antigen B27) - Pcr, Angiotensin Converting Enzyme, Anti-Nuclear Antibody (Ana) By Ifa - End Point Titer, Mpo (P-Anca) +11 More

| Test Name | Result | Bio. Ref. Interval |
|---|---|---|
| Inflammatory markers | 18 Feb 26 | |
| C-Reactive Protein (Quantitative) | 4.34 mg/L | 0 - 4.9 |
| Erythrocyte Sedimentation Rate | 5 mm/hr | 0 - 12 |
| Diabetes Profile | 18 Feb 26 | |
| Glucose - Fasting | 88 mg/dL | 70 - 99 |
| Kidney Health | 18 Feb 26 | |
| Creatinine | 0.60 mg/dL | 0.5 - 1 |
| Cardiac Profile | 18 Feb 26 | |
| **Homocysteine** | **44.28** Âµmol/L | 4.44 - 13.56 |
| Liver Health | 18 Feb 26 | |
| **Bilirubin - Total** | **0.26** mg/dL | 0.3 - 1.2 |
| **Bilirubin-Indirect** | **0.12** mg/dL | 0.2 - 0.8 |
| Protein, Total | 6.85 g/dL | 6.4 - 8.3 |
| Albumin | 4.43 g/dL | 3.5 - 5.2 |
| Aspartate Transaminase (SGOT) | 16 U/L | 11 - 34 |

**Trends** (For last three tests)

Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!

# Doctor Summary

Pr3(C-Anca),Hla B27 (HumanLeukocyte Antigen B27) - Pcr, Angiotensin Converting Enzyme, Anti-Nuclear Antibody (Ana) By Ifa - End Point Titer, Mpo (P-Anca) +11 More

| Test Name | Result | Bio. Ref. Interval |
|---|---|---|
| Liver Health | 18 Feb 26 | |
| Alanine Transaminase (SGPT) | 13 U/L | 0 - 34 |
| Alkaline Phosphatase | 112 U/L | 46 - 122 |
| Gamma Glutamyltransferase (GGT) | 10 U/L | 0 - 37.9 |
| Calcium and Bone Health | 18 Feb 26 | |
| **Vitamin D (25-OH)** | **29.2** ng/mL | 30 - 100 |
| Calcium | 9.3 mg/dL | 9.0 - 10.1 |
| Vitamins & Minerals | 18 Feb 26 | |
| **Vitamin B12** | **< 148** pg/mL | 187 - 833 |
| Arthritis Screening | 18 Feb 26 | |
| Anti-CCP | 0.50 U/mL | <= 4.99 |
| Rheumatoid Factor - Quantitative | < 20.0 0 IU/mL | 0 - 29.9 |
| Immuno-profile | 18 Feb 26 | |
| **ANA HEp-2** | **Positiv e** | Negative |

**Trends** (For last three tests)

Book more tests with Tata 1mg to start viewing trends. Your health journey starts here!

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. JANE DOE | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : OSNSIDBUDJS | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724484 / 15900187 | Report Date | : 18/Feb/2026 02:46PM |
| Sample Type | : Whole Blood-EDTA | Report Status | : Partial Report |

## *HAEMATOLOGY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Complete Blood Count*** | | | | |
| Hemoglobin | 12.4 | g/dL | 12.0 - 15.0 | Spectrophotometr y (Cyanide-free) |
| RBC | ***5.12*** | mili/cu.mm | 3.8 - 4.8 | Impedence |
| HCT | 37.9 | % | 36 - 46 | Calculated |
| MCV | ***74.1*** | fL | 83 - 101 | Calculated |
| MCH | ***24.2*** | pg | 27 - 32 | Calculated |
| MCHC | 32.6 | g/dL | 31.5 - 34.5 | Calculated |
| RDW-CV | ***17.5*** | % | 11.5-14 | Calculated |
| Total Leucocyte Count | 7.10 | 10^3/µL | 4 - 10 | Impedance |
| ***Differential Leucocyte Count*** | | | | |
| Neutrophils | 5 | % | 40-80 | DHSS/Microscopy |
| Lymphocytes | 9 | % | 20-40 | DHSS/Microscopy |
| Monocytes | 2 | % | 2-10 | DHSS/Microscopy |
| Eosinophils | 8 | % | 1-6 | DHSS/Microscopy |
| Basophils | 6 | % | 0-2 | Impedance / |
| ***Absolute Leucocyte Count*** | ***7*** | | | Microscopy |
| Absolute Neutrophil Count | 04.19 | 10^3/ | 2 - 7 | Calculated |
| Absolute Lymphocyte Count | 1.99 | µL | 1-3 | Calculated |
| Absolute Monocyte Count | 0.43 | 10^3/ | 0.2 - 1 | Calculated |
| Absolute Eosinophil Count | 0.5 | µL | 0.02 - 0.5 | Calculated |
| Absolute Basophil Count | ***0*** | 10^3/ | 0.02-0.1 | Calculated |
| Platelet Count | 328 | µL | 150-410 | Impedance/Microscop |
| MPV | 8 | 110^3/ | 6.5 - 12 | y Calculated |
| PDW | 13.1 | µL | 9 - 17 | Calculated |
| | | 110^3/ | | |
| | | µL | | |
| | | 10^3/ | | |
| | | µL | | |

***Comment:***

Asperthe recommendation of International council for Standardization in Hematology, the differential leucocyte counts are additionally being reported as absolute numbers of each cell in per unit volume of blood.
DHSS : Double Hydrodynamic Sequential System Flowcytometry
Calculated parameters are either derived from Impedence measure, RBC pulse measurement, RBC/platelet histograms or formula derived.

NABL certificate and scope

This test has been performed at
TATA 1MG PUNE
Address: Teerth Business Center, Ground Floor, Unit No 1, Sub-Divided Plot No EL-15, Bhosari MIDC, Pune - 411026

Dr. Sangeeta Parmar
(M.D., Pathologist)
Reg. No. 61255

Scan for digital copy



State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Bangalore · Hyderabad
Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724484 / 15900187 | Report Date | : 18/Feb/2026 03:37PM |
| Sample Type | : WHOLE BLOOD-EDTA | Report Status | : Partial Report |

## *HAEMATOLOGY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Erythrocyte Sedimentation Rate*** | | | | |
| Erythrocyte Sedimentation Rate | 5 | mm/hr | 0-12 | Modified Westergren |

***Comment:***

- ESR provides an index of progress of the disease and is widely used as an indicator of inflammation, infection, trauma, or malignant diseases. Changes are more significant than a single abnormal test
- It is specifically indicated to monitor the course or response to the treatment of diseases like rheumatoid arthritis, tuberculosis bacterial endocarditis ,acute rheumatic fever ,Hodgkins disease,temporal arthritis , and systemic lupus erythematosis; and to diagnose and monitor giant cell arteritis and polymyalgia rheumatica.
- An elevated ESR may also be associated with many other conditions, including autoimmune disease, anemia, infection,malignancy,pregnancy, multiple myeloma, menstruation, and hypothyroidism.
- Although a normal ESR cannot be taken to exclude the presence of organic disease, its rate is dependent on various physiologic and pathologic factors.
- The most important component influencing ESR is the composition of plasma. High level of C-Reactive Protein, fibrinogen, haptoglobin, alpha-1antitrypsin, ceruloplasmin and immunoglobulins causes the elevation of Erythrocyte Sedimentation Rate.
- Drugs that may cause increase ESR levels include: dextran, methyldopa, oral contraceptives, penicillamine, procainamide, theophylline, and Vitamin A. Drugs that may cause decrease levels include: aspirin, cortisone, and quinine

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724483 / 15900187 | Report Date | : 18/Feb/2026 03:58PM |
| Sample Type | : Fluoride Plasma F | Report Status | : Partial Report |

## *BIOCHEMISTRY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***FBS (Fasting Blood Sugar)*** | | | | |
| Glucose - Fasting | 88 | mg/dL | 70 - 99 | Hexokinase/ G-6-PDH |

| Fasting Plasma Glucose (mg/dL) | 2 hr plasma Glucose (mg/dL) | Diagnosis |
|---|---|---|
| 99 or below | 139 or below | Normal |
| 100 to 125 | 140 to 199 | Pre-Diabetes (IGT) |
| 126 or above | 200 or above | Diabetes |

Reference : American Diabetes Association

***Comment:***

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes.
IGT (2 hrs Post meal ), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes

| Plasma Glucose Goals | For people with Diabetes |
|---|---|
| Before meal | 70-130 mg/dL |
| 2 Hours after meal | Less than 180 mg/dL |
| HbA1c | Less than 7% |

NABL certificate and scope

This test has been performed at
*TATA 1MG PUNE*
Address: Teerth Business Center, Ground Floor, Unit No 1, Sub-Divided Plot No EL-15, Bhosari MIDC, Pune - 411026

Dr. Naziya Balechand Maner
MBBS, DCP, DNB (Pathology)
Consultant Pathologist
Reg No: 2007/05/1618

Scan for digital copy



State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Bangalore · Hyderabad
Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 01:30PM |
| Barcode ID/Order ID | : D29727237 / 15900187 | Report Date | : 18/Feb/2026 06:56PM |
| Sample Type | : Fluoride Plasma P | Report Status | : Partial Report |

## *BIOCHEMISTRY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***PPBS (Postprandial Blood Sugar)*** | | | | |
| Glucose Postprandial | 102 | mg/dL | 70 - 140 | Hexokinase/ G-6-PDH |

***Comment:***

Impaired glucose tolerance (IGT) fasting, means a person has an increased risk of developing type 2 diabetes but does not have it yet. A level of 126 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes.
IGT (2 hrs Post meal ), means a person has an increased risk of developing type 2 diabetes but does not have it yet. A 2-hour glucose level of 200 mg/dL or above, confirmed by repeating the test on another day, means a person has diabetes.

| Plasma Glucose Goals | For people with Diabetes |
|---|---|
| Before meal | 70-130 mg/dL |
| 2 Hours after meal | Less than 180 mg/dL |
| HbA1c | Less than 7% |

TATA 1mg | Labs

NABL certificate and scope

This test has been performed at
*TATA 1MG PUNE*
Address: Teerth Business Center, Ground Floor, Unit No 1, Sub-Divided Plot No EL-15, Bhosari MIDC, Pune - 411026

Dr. Naziya Balechand Maner
MBBS, DCP, DNB (Pathology)
Consultant Pathologist
Reg No: 2007/05/1618

Scan for digital copy



State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Bangalore · Hyderabad
Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 18/Feb/2026 04:46PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *BIOCHEMISTRY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***LIVER FUNCTION TEST*** | | | | |
| ***Liver Function Test*** Bilirubin-Total | ***0.26*** | mg/d | 0.3-1.2 | Diazonium Salt |
| Bilirubin-Direct | 0.14 | L | 0-0.5 | Diazo |
| Bilirubin-Indirect | ***0.12*** | mg/d | 0.2-0.8 | Calculated |
| Protein, Total | 6.85 | L | 6.4-8.3 | Biuret |
| Albumin | 4.43 | mg/d | 3.5-5.2 | Bromocresol Green |
| Globulin | 2.4 | L | 2.3 - 4.1 | Calculated |
| A/G Ratio | 1.83 | g/dL | 0.8 - 1.9 | Calculated |
| SGOT (Aspartate | 16 | g/dL | 11-34 | NADH w/o P-5'-P |
| Aminotransferase) SGPT (Alanine | 13 | g/dL | 0-34 | NADH w/o P-5'-P |
| Transaminase) SGOT/SGPT | 1.19 | Ratio | | Calculated Para- |
| Alkaline Phosphatase | 112 | U/L U/L | 46-122 | Nitrophenyl Phosphate |
| Gamma Glutamyltransferase (GGT) | 10 | Ratio U/L | <38 | L-gamma-glutamyl-3-Carboxy-4-Nitroanilide |

***Comment:***

- Raised ALT and AST indicate hepatocellular damage (e.g. viral or drugs etc). ALT is more liver-specific while AST is also found in heart, skeletal muscle, and kidney. Mild elevation (less than twice normal) often resolves on its own. Fatty liver disease (especially with metabolic syndrome) is a common cause in asymptomatic cases. Certain drugs (paracetamol, statins), herbal supplements, energy drinks, and antibiotics may also affect liver function.
- SGOT/SGPT Ratio: Typically <1 in healthy individuals (vary between 0.7-1.4; higher in women than men). High SGPT (ratio <1) seen in acute or chronic hepatitis, autoimmune disorders, medications, toxins while ratio >1 indicates alcoholic hepatitis, cirrhosis, metastasis or non-hepatic issues (hemolytic diseases, CVS disorders).
- Elevated Alkaline Phosphatase and GGT: Suggest cholestatic diseases (e.g. bile duct obstruction, primary biliary cirrhosis etc.) and can also be due to bone disease, pregnancy, chronic renal failure, malignancy, and congestive heart failure.
- High Bilirubin: Indicates jaundice due to increased RBC breakdown, liver damage (e.g., infections, toxins), or cholestasis (e.g., gallstones, tumors).
- High Protein Levels: Seen in dehydration (e.g., severe vomiting, diarrhea) or increased production (e.g., inflammation, hematopoietic neoplasms). Low protein and albumin: Result from impaired synthesis (liver disease), decreased intake,

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 18/Feb/2026 04:46PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *BIOCHEMISTRY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|

tissue damage, malabsorption, or increased renal excretion.

***Creatinine***

| | | | | |
|---|---|---|---|---|
| Creatinine | 0.60 | mg/dL | 0.5 - 1.0 | Kinetic Alkaline Picrate |

***Comment:***

- Creatinine is a more specific and sensitive indicator of renal disease than Blood Urea Nitrogen.

Uses:

- To diagnose renal insufficiency;
- Adjusting dosage of renally excreted medications.
- Monitoring renal transplant recipients.
- Serum creatinine levels are a proxy for reduced skeletal muscle mass.
- Serum creatinine measurement is used in estimating the Glomerular Filtration Rate (GFR) for people with Chronic Kidney disease (CKD) and those with risk factors for CKD (Diabetes Mellitus, hypertension, cardiovascular disease, and family history of kidney disease).

Increased In: Blockage in the urinary tract, Pre- and postrenal azotemia, Impaired kidney function, Loss of body fluid (dehydration), Muscle diseases such as gigantism, acromegaly.
Decreased In: Pregnancy, certain drugs (e.g., cimetidine, trimethoprim), Myasthenia Gravis, Muscular dystrophy.

NABL certificate and scope

This test has been performed at
*TATA 1MG PUNE*
Address: Teerth Business Center, Ground Floor, Unit No 1, Sub-Divided Plot No EL-15, Bhosari MIDC, Pune - 411026

Dr. Naziya Balechand Maner
MBBS, DCP, DNB (Pathology)
Consultant Pathologist
Reg No: 2007/05/1618

Scan for digital copy



State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Bangalore · Hyderabad
Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 18/Feb/2026 04:46PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *BIOCHEMISTRY*

### *VITAMIN D, B12 & CALCIUM*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Calcium*** | | | | |
| Calcium | 9.3 | mg/dL | 9.0 - 10.1 | Arsenazo III |

***Comment:***

Increasedin: Hyperparathyroidism primary and secondary, Acute and chronic renal failure, Following renal transplantation, Osteomalacia with malabsorption, Acute osteoprosis, Malignant tumours (specially of breast, lung and kidney), Drugs: Vit. D and A intoxication, Diuretics, estrogen, androgen, tamoxifen, lithium

Decreased in: Hypoparathyroidism, Surgical and Idiopathic, Pseudohypoparathyroidism, Chronic renal disease with uremia and phophate retention, Malabsorption of Calcium and Vit.D, obstructive jaundice, Bone Disease ( Osteomalacia and rickets), Drugs: Cancer chemotherapy drugs, calcitonin, loop-actives diuretics, Hypomagnesemia,Hypoalbuminemia

TATA 1mg | Labs

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 18/Feb/2026 04:46PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *BIOCHEMISTRY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***C-Reactive Protein Quantitative*** | | | | |
| C-Reactive Protein (Quantitative) | 4.34 | mg/L | <5.0 | Immunoturbidimetric |

***Comment:***
•C-Reactive Protein [CRP] is an acute phase reactant ,hepatic secretion of which is stimulated in response to inflammatory cytokines.
•CRP is a very sensitive but nonspecific marker of inflammation and infection.
•The CRP test is useful in patient with Inflammatory bowel disease, arthritis, Autoimmune diseases, Pelvic inflammatory disease (PID), tissue injury or necrosis and infections.
•CRP levels can be elevated in the later stages of pregnancy as well as with use of birth control pills or hormone replacement therapy i.e. estrogen. Higher levels of CRP have also been observed in the obese.
•As compared to ESR, CRP shows an earlier rise in inflammatory disorders which begins in 4-6 hrs, he intensity of the rise being higher than ESR and the recovery being earlier than ESR. Unlike ESR, CRP levels are not influenced by hematologic conditions like Anemia, Polycythemia.

TATA 1mg Labs

| | | | | |
|---|---|---|---|---|
| ***Rheumatoid Factor - Quantitative*** | | | | |
| Rheumatoid Factor - Quantitative | < 20.00 | IU/mL | < 30 Negative,<br>30 - 50 Weakly Positive,<br>> 50 Positive | Turbidimetry |

***Comment:***

- The detection of Rheumatoid factor (RF) is one of the criteria of the American Rheumatism Association (ARA) for the diagnosis of Rheumatoid Arthritis (RA).
- RF are heterogeneous group of auto antibodies directed against Fc- region of IgG molecules.
- They are useful in diagnosis of Rheumatoid Arthritis, but can also be found in other inflammatory diseases and in various non-rheumatic diseases.
- These occur in all the immunoglobulin classes, although the usual analytical methods are limited to the detection of Rheumatoid Factors of the IgM type.Healthy individuals >65 years of age may also show positive RF results.

NABL certificate and scope

This test has been performed at
*TATA 1MG PUNE*
Address: Teerth Business Center, Ground Floor, Unit No 1, Sub-Divided Plot No EL-15, Bhosari MIDC, Pune - 411026

NRanee
Dr. Naziya Balechand Maner
MBBS, DCP, DNB (Pathology)
Consultant Pathologist
Reg No: 2007/05/1618

Scan for digital copy



State of the Art Lab Facilities
Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Bangalore · Hyderabad
Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 20/Feb/2026 09:03AM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *BIOCHEMISTRY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Angiotensin Converting Enzyme (ACE)*** | | | | |
| Angiotensin converting enzyme | 35.07 | U/L | > or =18 years: 8-52<br>Paediatric = upto 50% higher than that of adults. | Enzymatic |

### *Comment:*

In Sarcoidosis, Angiotensin-converting enzyme (ACE) is produced by epithelioid cells derived from activated macrophages and is a known marker for sarcoidosis. ACE level correlates with the amount of whole-body granulomas (more commonly lungs) and illness severity.

Decreased ACE levels are seen in people with Chronic obstructive pulmonary disease (COPD), Lung diseases such as emphysema, lung cancer, cystic fibrosis, Starvation, Steroid drug therapy, Hypothyroidism.

Moderately increased levels are seen in a variety of diseases and disorders, such as HIV, Histoplasmosis, Diabetes mellitus, Hyperthyroidism, Lymphoma, Alcoholic cirrhosis, Gaucher disease, Tuberculosis, Leprosy.

Note:

- Normal, healthy children and infants are known to have ACE activity levels greater than the adult reference interval.
- Serum ACE is significantly reduced in patients on ACE inhibitors.
- A single ACE measurement does not predict the presence or extent of lung injury, or aid in diagnosis. However, serial levels are of value prognostically.

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 18/Feb/2026 05:11PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *IMMUNOLOGY*

### *VITAMIN D, B12 & CALCIUM*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Vitamin D (25-OH)*** | | | | |
| Vitamin D (25-OH) | ***29.2*** | ng/mL | Deficiency:< 20,<br>Insufficiency:20-29,<br>Sufficiency:30 - 100,<br>Toxicity possible:> 100 | CMIA |

***Comment:***

- Vitamin D is a fat-soluble steroid prohormone involved in the intestinal absorption of calcium and the regulation of calcium homeostasis.
- Two forms of vitamin D are biologically relevant - vitamin D3 (Cholecalciferol) and vitamin D2 (Ergocalciferol).
- Both vitamins D3 and D2 can be absorbed from food but only an estimated 10-20perc. of vitamin D is supplied through nutritional intake.
- Vitamin D is converted to the active hormone 1,25-(OH)2-vitamin D (Calcitriol) through two hydroxylation reactions. The first hydroxylation converts vitamin D into 25-OH vitamin D and occurs in the liver. The second hydroxylation converts 25-OH vitamin D into the biologically active 1,25-(OH)2-vitamin D and occurs in the kidneys as well as in many other cells of the body.
- Most cells express the vitamin D receptor and about 3perc. of the human genome is directly or indirectly regulated by the vitamin D endocrine system.
- The major storage form of vitamin D is 25-OH vitamin D and is present in the blood at up to 1,000 fold higher concentration compared to the active 1,25-(OH)2-vitamin D. 25-OH vitamin D has a half-life of 2-3 weeks vs. 4 hours for 1,25-(OH)2-vitamin D. Therefore, 25-OH vitamin D is the analyte of choice for determination of the vitamin D status.
- Risk factors for vitamin D deficiency include low sun exposure, inadequate intake, decreased absorption, abnormal metabolism, vitamin D resistance and and liver or kidney diseases.
- Vitamin D deficiency is a cause of secondary hyperparathyroidism and diseases resulting in impaired bone metabolism (like rickets, osteomalacia).
- Recently, many chronic diseases such as cancer, high blood pressure, osteoporosis and several autoimmune diseases have been linked to vitamin D deficiency.
- The assay measures both D2 (Ergocalciferol) and D3 (Cholecalciferol) metabolites of vitamin D

Utility Quantitative determination of 25-hydroxyvitamin D (25-OH vitamin D).

* ***CMIA***-Chemiluminescent Microparticle Immunoassay /***CLIA***-Chemiluminescent immunoassay.

NABL certificate and scope

This test has been performed at
*TATA 1MG PUNE*
Address: Teerth Business Center, Ground Floor, Unit No 1, Sub-Divided Plot No EL-15, Bhosari MIDC, Pune - 411026

Dr. Naziya Balechand Maner
MBBS, DCP, DNB (Pathology)
Consultant Pathologist
Reg No: 2007/05/1618

Scan for digital copy

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 18/Feb/2026 05:11PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *IMMUNOLOGY*

### *VITAMIN D, B12 & CALCIUM*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Vitamin B12*** | | | | |
| Vitamin B12 | ***< 148*** | pg/mL | 187 - 833 | CMIA |

***Comment:***

- Vitamin B12 along with folate is essential for DNA synthesis and myelin formation.
- Decreased levels are seen in anaemia, term pregnancy, vegetarian diet, intrinsic factor deficiency, partial gastrectomy/ileal damage, celiac disease, oral contraceptive use, parasitic infestation, pancreatic deficiency, treated epilepsy, smoking, hemodialysis and advanced age.
- Increased levels are seen in renal failure, hepatocelluar disorders, myeloproliferative disorders and at times with excess supplementation of vitamins pills.

* ***CMIA***-Chemiluminescent Microparticle Immunoassay /***CLIA***-Chemiluminescent immunoassay.

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 18/Feb/2026 05:11PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *IMMUNOLOGY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Homocysteine*** | | | | |
| Homocysteine | ***44.28*** | µmol/L | 4.44-13.56 | CMIA |

***Comment:***

Interpertation:
Increased levels are seen in deranged Vit B12 metabolism and form an independent marker for risk of thromboembolic episodes in coronary artery disease (CAD)

Clinical Utility:
•Determine risk for heart disease, stroke and peripheral arterial blood vessel disease.
•Identify vitamin B12 deficiency or folic acid deficiency.
•Identify homocystinuria

The recommended use of Homocysteine (HCY) to assess risk factor for CAD are
•It is specially useful in young CAD patients (<40 years)
•In known cases of CAD,high HCYlevels should be used as a prognostic marker for CAD events and mortality.
•CAD patients with HCY levels >15 umol/L belong to high risk group.
•Increased HCY levels with low vitamin concentrations should be handled as a potential vitamin deficiency case .

High values of HCY are found in dietary deficiency of folic acid, vitamin B6, or vitamin B12, homocystinuria, chronic liver and renal failure,post menopausal state , hypothyroidism, Alzheimer's disease,vaious neoplastic disease like cancers of ovary or breast and Acute lymphoblastic leukemia,drugs (anti-anticonvulsants, antibiotics, theophylline, birth control pills, and tamoxifen),alcoholism, smoking or tobacco usage.
Low values may be caused by some medicines or vitamins such as folic acid, vitamin B12, or niacin.

• Please note test values may vary depending on the assay method used.
• CMIA-Chemiluminescent Microparticle Immuno Assay

* ***CMIA***-Chemiluminescent Microparticle Immunoassay /***CLIA***-Chemiluminescent immunoassay.

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 18/Feb/2026 05:11PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *IMMUNOLOGY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Anti Cyclic Citrullinated Peptide*** | | | | |
| (Anti-CCP) | 0.50 | U/mL | negative : < 5.0 U/mL<br>positive : >= 5.0 U/mL | CMIA |

***Comment:***

- Anti-CCP is semi quantitative determination of Immunoglobulin G auto antibodies specific to Cyclic citrullinated peptide.
- It is useful in the diagnosis of Rheumatoid arthritis(RA), with high specificity, presence early in the disease process and ability to identify patients who are likely to have severe disease and irreversible change.
- False positive results are uncommon. Up to 30% patient with seronegative Rheumatoid arthritis also show Anti-CCP antibodies.
- The sensitivity of this assay is 70.6% and specificity is 98.2%.
- The value of Anti-CCP in Juvenile arthritis patients has not been determined.
- Anti CCP antibodies are detected in approximately 50-60% patient of Rheumatoid arthritis usually after 3-6 months of symptoms.
- Early Rheumatoid arthritis patients with Anti-CCP positivity may develop a more erosive form of the disease as compared with Anti-CCP negative patients.
- It can be used to differentiate elderly onset Rheumatoid arthritis from Polymyalgia rheumatica and erosive SLE.

* ***CMIA***-Chemiluminescent Microparticle Immunoassay /***CLIA***-Chemiluminescent immunoassay.

NABL certificate and scope

This test has been performed at
*TATA 1MG PUNE*
Address: Teerth Business Center, Ground Floor, Unit No 1, Sub-Divided Plot No EL-15, Bhosari MIDC, Pune - 411026

Dr. Naziya Balechand Maner
MBBS, DCP, DNB (Pathology)
Consultant Pathologist
Reg No: 2007/05/1618

Scan for digital copy



State of the Art Lab Facilities

Delhi · Gurgaon · Jaipur · Lucknow · Dehradun · Prayagraj · Mumbai · Pune · Ahmedabad · Bangalore · Hyderabad
Chennai · Visakhapatnam · Kolkata · Bhubaneswar · Angul · Patna · Ranchi · Guwahati

TATA 1mg | Labs
Tata 1mg Technologies Private Limited
Registered office: 2nd Floor, Plot No. B-225, Okhla Industrial Area, Phase-I, South Delhi, New Delhi-110020, Delhi
www.1mg.com/labs care@1mg.com CIN: U74140DL2015PTC279229
ISO 9001:2015

CAP ACCREDITED
COLLEGE of AMERICAN PATHOLOGISTS

Most Trusted Brand
40 lakhs+
satisfied customers
1 crore+
lab tests booked

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 19/Feb/2026 06:18PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *SEROLOGY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***Anti-Nuclear Antibody (ANA) by IFA - End Point Titer*** | | | | |
| ANA HEp-2 | ***Positive*** | | Negative | IFA |
| Pattern. | Nucleolar | | | |
| Intensity. | + | | | |
| End Point Titer | 1:80 | | | |

***Comment:***
Interpretation Guidelines: (Sample screening Dilution: 1:80)
Negative : No Immunofluorescence
+ Weak Positive (1:80)
++ : Moderate Positive (1:160)
+++ : Strong Positive (1:320)
++++ : Very strong Positive (1:640)

ANA by IFA (Immunofluorescence Assay) using HEp 2000 uses a genetically modified HEp 2 cell line with increased sensitivity for Anti SSA(Ro) which is missed on the conventional HEp 2 cells, without affecting the other IFA (Immunofluorescence Assay) patterns.

| Location | Pattern | Target Antigen | Clinical Association |
|---|---|---|---|
| Nucleus | Homogeneous | Double strand DNA<br>Histones<br>Nucleosome, RNA, Single Strand DNA | SLE Drug Induced Lupus, SLE<br>RA SLE, MCTD, RA, PM, DM,<br>SS |
| | Speckled | Sm<br>U1-snRNP<br>SSA/Ro<br>SSB/La<br>Ku<br>Cyclin1(PCNA)<br>Mitosin/Cyclin II | SLE<br>MCTD,SLE, RA, sharp syndrome<br>Sjogren's<br>syndromes(SS)/SLE/Neonatal Lupus PM/DM/SLE/SS<br>SLE/Overlap Syndromes<br>DM |
| | Dense Fine Speckled(DFS) | Lens epithelium-derived growth factor (LEDGF), DNA binding transcription coactivator p75.(DFS-70) | Healthy individuals, Various Inflammatory conditions like atopic dermatitis, interstitial cystitis , Asthma. |
| | Centomeres | Proteins of Kinetochores | CREST syndrome, PSS limited form |
| | Nuclear Dots | Sp-100, NDP53 | PBC, Rheumatic Disease |
| | Nuclear Membrane | Lamins, gp210, p62 | CFS, Collagenoses, PBC, AIH |
| Nucleolus | Nucleolar homogeneous | PM-Scl<br>Scl-70 | PM, DM, PSS(Diffuse)<br>PSS(Diffuse) |

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 19/Feb/2026 06:18PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *SEROLOGY*

| *Test Name* | | *Result* | *Unit* *Bio. Ref. Interval* *Method* |
|---|---|---|---|
| | Nucleolar speckled | RNA-Polymerase I / NOR-90 | Progressive Systemic Sclerosis(Diffuse) |
| | Nucleolar Pattern | Fibrillarin | Progressive Systemic Sclerosis(Diffuse) |
| Cytoplasm | Cytoplasmic speckled | Mitochondrial<br>Lysosomal<br>Golgi Complex<br>Ribosome P<br>Jo -1<br>SRP, PL12, TIF1-Gamma | PBC,<br>Unknown<br>SS/SLE/RA<br>SLE Polymyositis (PM),<br>PM/ DM, Myositis |
| | Cytoplasmic filament | F- Actin<br>Vimentin<br>Tropomyosin<br>Cytoplasmic Rings & rods | AIH<br>Unknown<br>Unknown<br>HCV Infection- on therapy |
| Cell Cycle (mitotic cells) | Centriole Mid-Body<br>Spindle Fibres | --<br>--<br>-- | Unknown<br>Unknown<br>Rheumatic Disease |

Note:

- ANA is reported in low titres in a significant proportion of the healthy population and results need to be correlated clinically. Autoantibodies may not always correlate with the observed pattern and confirmatory tests for positive results are recommended where available. ANA results should only be interpreted by the clinician in light of the patient's clinical information. An ANA titer and ANA profile is suggested for ANA positive results.
- All weak positive (+) results may be repeated after 6 - 8 weeks.
- Associated Tests: Monospecific ELISA to define single antigens, ANA Immunoblot assay.

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 20/Feb/2026 08:21PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *SEROLOGY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***C ANCA Serine protease 3 (PR-3) Antibodies*** | | | | |
| PR - 3 | 0.72 | Unit | <=20 | EIA |

***Comment:***

| | |
|---|---|
| <=20 | Negative: No PR-3 antibody or levels below negative cutoff |
| 21 - 30 | Weak positive |
| >30 | Moderate positive to strong positive: Suggests possibility of certain autoimmune vasculitidis |

Anti-neutrophil cytoplasmic antibody (ANCA) testing is widely used for diagnosis and treatment of various autoimmune mediated vasculitidis. The p-ANCA and c-ANCA auto antibodies are shown to be clinically correlated to diseases such as Wegener's granulomatosis (WG) and crescentic glomerulonephritis.
This test helps in differentiating true ANA from granulocyte specific ANA which may occur concurrently & immunofluorescence methods may not adequately make this distinction. Hence a combination of Immunofluorescence & EIA methods are recommended for confirming presence of PR3 antibody / c-ANCA.

Note:

- Results should be used in conjugation with clinical finding and other serological tests including ANCA by indirect immunofluorescence.
- This assay to detects IgG class antibodies against serine protease 3 (PR-3) in human serum samples.
- Presence of Anti- PR3 IgG antibodies suggests the possibility of certain autoimmune vasculitidis such as Wegener's granulomatosis (WG).
- The presence of immune complexes or other immunoglobulin aggregates in the patient sample may cause an increased level of non specific binding and produce false positive results. Thus, it is suggested not to initiate immunosuppressive therapy based solely on this test positive results.
- The values of these antibodies are significantly higher during active phases of diseases.

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724481 / 15900187 | Report Date | : 19/Feb/2026 02:50PM |
| Sample Type | : Serum | Report Status | : Partial Report |

## *SEROLOGY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| ***P-ANCA Anti-Myeloperoxidase (MPO) antibodies*** | | | | |
| Anti - MPO | 1.21 | Unit | <=20 | EIA |

***Comment:***

| | |
|---|---|
| <=20 | Negative: No MPO antibody or levels below negative cutoff |
| 21-30 | Weak positive |
| >30 | Moderate positive to strong positive: Suggests possibility of certain autoimmune vasculitis |

Anti-neutrophil cytoplasmic antibody (ANCA) testing is widely used for diagnosis and treatment of various autoimmune mediated vasculitidis. The p-ANCA and c-ANCA auto antibodies are shown to be clinically correlated to diseases such as Wegener's granulomatosis (WG) and crescentic glomerulonephritis. Anti- MPO IgG antibodies are highly specific for idiopathic and vasculitis associated crescentic glomerulonephritis and also for classic polyarteritis nodosa. This test helps in differentiating true ANA from granulocyte specific ANA which may occur concurrently & immunofluorescence methods may not adequately make this distinction. Hence a combination of Immunofluorescence & EIA methods are recommended for confirming presence of MPO antibody / p-ANCA.

Note:

- Results should be used in conjugation with clinical finding and other serological tests including ANCA by indirect immunofluorescence.
- This assay detects IgG class antibodies against Myeloperoxidase (MPO) in human serum samples.
- Presence of Anti- MPO IgG antibodies suggests the possibility of certain autoimmune vasculitidis such as polyarteritis nodosa and crescentic glomerulonephritis.
- The values of these antibodies are significantly higher during active phases of diseases.
- The presence of immune complexes or other immunoglobulin aggregates in the patient sample may cause an increased level of non specific binding and produce false positive results. Thus, it is suggested not to initiate immunosuppression therapy based solely on this test positive results.

PO No :PO10001375239-194

| | | | |
|---|---|---|---|
| Customer Name | : Ms. | Collected Via | : TATA 1MG PUNE |
| Age/Gender | : 31/Female | Referred By | : Dr. |
| Lab Visit ID | : | Collection Date | : 18/Feb/2026 10:55AM |
| Barcode ID/Order ID | : D29724484 / 15900187 | Report Date | : 20/Feb/2026 02:45PM |
| Sample Type | : WHOLE BLOOD-EDTA | Report Status | : Partial Report |

## *MOLECULAR BIOLOGY*

| *Test Name* | *Result* | *Unit* | *Bio. Ref. Interval* | *Method* |
|---|---|---|---|---|
| HLA B27 PCR | NOT DETECTED | | | Real Time PCR |

***Comment:***
Clinical Background:
HLA-B27 RT PCR is a test for qualitative analysis of detection of HLA-B27 antigen expressions present on cell surfaces. HLA-B27 is a major histocompatibility complex (MHC) class I molecule. MHC class I molecules are cell-surface glycoproteins that are expressed on most nucleated human cells and platelets. The presence of HLA-B27 antigen is strongly associated with ankylosing spondylitis (AS), a chronic inflammatory disease of the axial musculoskeletal system and a few other rheumatic disorders (Reiter’s syndrome, acute anterior uveitis, and inflammatory bowel disease).
HLA- B27 testing is routinely used to screen for AS patients.

TEST DONE BY TRUPCR® HLA-B27 REAL TIME PCR (IVD APPROVED) Kit.

***Note:*** Report to be interpreted in conjunction with clinical radiological & other lab findings.

**** End Of Report ****

*Pending Test-Quantiferon-TB Gold plus (Interferon Gamma Release Assay)*

Disclaimer:
1. The reported results based on laboratory investigation, are only for the purposes of diagnosis and should be clinically correlated and interpreted by the referring physician/ medical practitioner. For any queries relating to the reported results, you may write to our customer support team on care@1mg.com
2. It is presumed that the tests performed are, on the specimen / sample being to the patient named or identified and the verifications of particulars have been confirmed by the patient or his / her representative at the point of generation of said specimen.
3. The reported results are restricted to the given specimen only. Results may vary from lab to lab and from time to time for the same parameter for the same patient (within subject biological variation).
4. The patient's details along with their results in certain cases like notifiable diseases and as per local regulatory requirements will be communicated to the assigned regulatory bodies.
5. The patient samples can be used as part of internal quality control, test verification, data analysis purposes within the testing scope of the laboratory.
6. This report is not valid for medico legal purposes. It is performed to facilitate medical diagnosis only.
7. Pregnant women should seek guidance from a qualified obstetrician as test parameters may vary during pregnancy

TATA 1mg | Labs

TATA

# ENSURING ACCURACY IN EVERY SINGLE REPORT

## Following a 3-step review process:

Advanced systems & cutting-edge technology analyze results with precision

Experienced lab experts and technicians conduct comprehensive reviews

Each report undergoes rigorous medical scrutiny & is signed off by a doctor

**Have concerns regarding the report?**
Reach out to our care team at **care@1mg.com** or **chat with us**

**CONNECT NOW ›**

## Accurate Testing, Assured Quality: Diagnostics Precision

**Stringent quality control**
Measuresmeeting international norms of safety guidelines

**Cutting-edge technology**
Robusthealthcaresystems equipped with calibrated & well-maintained machinery

**Experienced lab staff**
30+ medicalprofessionals with a collective experience of 200+ years

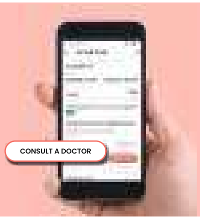
**Verified test procedures**
Highly standardized test procedures following CLSI* guidelines

**External assessments**
Thorough third-party assessment by authorized experts

**Trained phlebotomists**
Ensuring smoothsample collection experience & pre-analytical precision


**Claim FREE doctor consultation**
Consult top doctors from the comfort of your home

**SCHEDULE NOW›**




**Watch how we take care of your sample**


TATA 1mg CANCER care
Making Strides In Compassionate Care

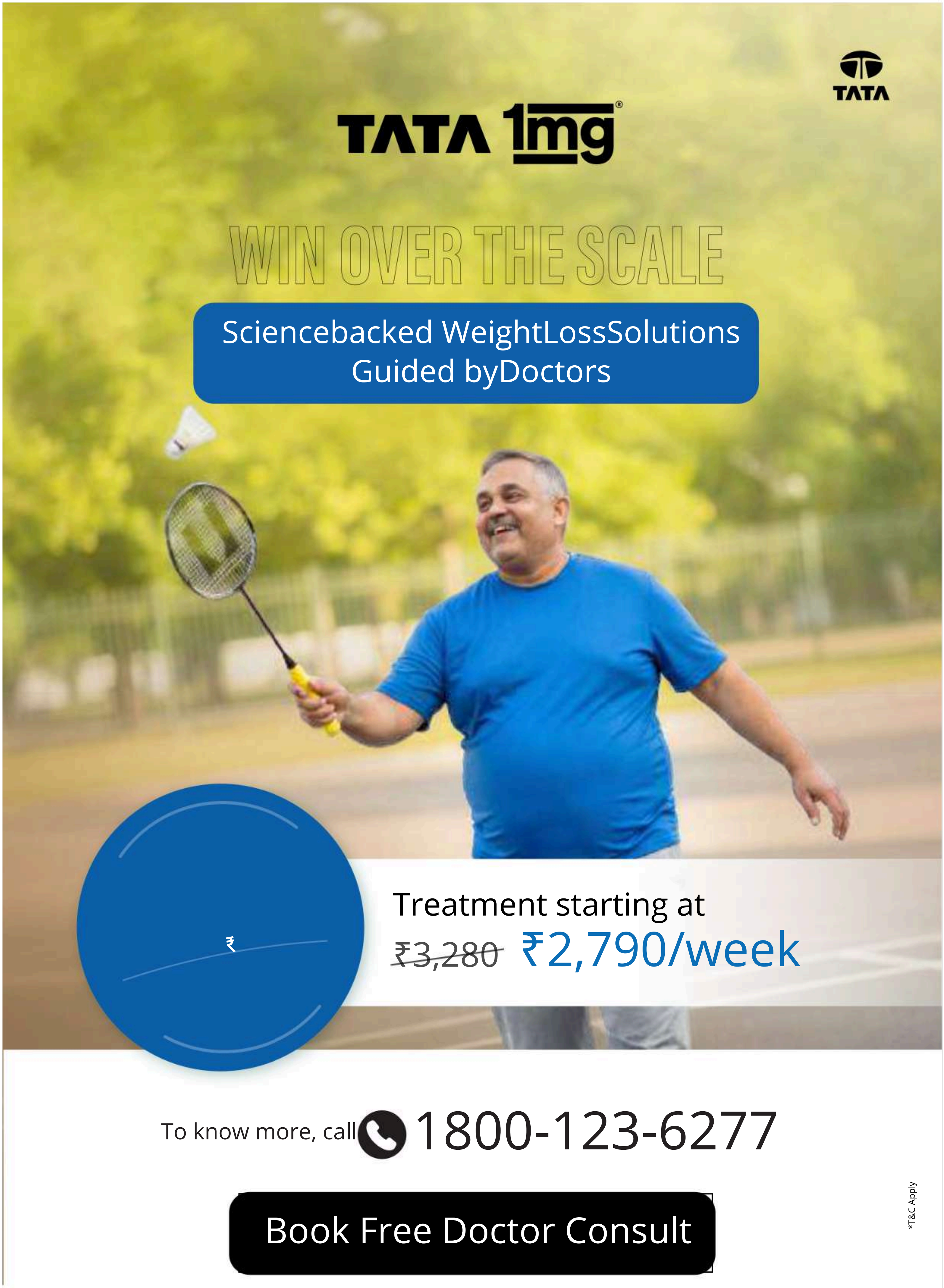
Get Second Opinion
Order Medicines
Book Lab Tests
Explore Financial Support & more

**EXPLORE NOW ›**

Need Help?
**1800-102-1618**


*Clinical & Laboratory Standards Institute

T&C Apply


TATA
TATA 1mg
WIN OVER THE SCALE
Sciencebacked WeightLossSolutions
Guided byDoctors
Treatment starting at
~~₹3,280~~ ₹2,790/week
To know more, call 1800-123-6277
Book Free Doctor Consult
*T&C Apply